

June 17, 2025

Habilitation Center, LLC
1810 Industrial Dr
Fordyce, AR 71742-7110

Arkansas Foundation for Medical Care (AFMC) performs Inspections of Care (IOC) reviews of the Behavioral Health Agencies for the Division of Provider Services and Quality Assurance with the Arkansas Department of Human Services. The Arkansas Department of Human Services Division of Behavioral Health Services Licensure Standards for Inpatient Psychiatric Services for Under 21. The Medicaid Manual for Inpatient Psychiatric Services for Under Age 21 was used in the completion of this report

An incident occurred at the following service site that prompted an order for an additional review. Upon review of the policies relevant to the incident, the findings are noted below:

Habilitation Center, LLC
Provider Medicaid ID: [REDACTED]
Onsite Inspection Date: June 2, 2025
Onsite Inspection Time: 9:02 AM

A summary of the inspection, policies reviewed, and findings are noted below.

Inspection of Care Summary

Health and Safety-Policy Review

This additionally ordered inspection was triggered by a complaint against Habilitation Center, LLC regarding a client's medications, late or missing labs, significant weight gain, the development of prediabetes or diabetes, and the lack of physical activity. Based on the nature of the incident, the following were reviewed and interviews conducted:

- Client file of the alleged victim
- Staff interviews
- Client/guardian interviews
- Employee File Review

Observation:

Upon arrival at the facility, AFMC staff were promptly greeted at the entrance by a Habilitation Center, LLC receptionist in the main lobby. AFMC staff signed the visitor log. AFMC staff were immediately taken to a conference room where they were met by the Chief Executive Officer, Director of Risk Management, Director of Clinical Services, and the Chief Nursing Officer. AFMC staff were given the completed and signed consent form listing approval for access to the AFMC portal. Facility staff were given the Document Request Form, and AFMC staff discussed the requirements for the Inspection of Care.

AFMC staff reviewed the final document request form with the Chief Executive Officer, Director of Risk Management, and the Director of Clinical Services at the completion of the on-site Inspection of Care and the

provider signed the acknowledgement of manual requirements that were not made available in the provider's policy and procedures. AFMC staff explained that these findings were preliminary, and there was still an off-site portion to be completed.

Facility Review – Policies and Procedures:

Upon review of the site's policies, procedures, and certification requirements, the following deficiencies were noted:

Regulation	Deficiency Statement	Reviewer Notes
Medicaid IP Sec. 2: 42 CFR 482.130, 42 CFR 483.376	HR records did not indicate that all direct care personnel are currently certified in cardiopulmonary resuscitation (CPR).	Upon review of employee SR020485, it was observed that no certification in cardiopulmonary resuscitation (CPR) was included.
Medicaid IP Sec. 2: 215.220, 218.000 42 CFR 441.156	There is no documentation that all direct care personnel hold current licenses, as required by their position and profession and/or licensing authority.	The provider lacked evidence of professional licenses for one of the reviewed professionals.

Personnel Records – Licenses, Certifications, Training:

There was a total of 50 personnel records reviewed, eleven (27%) professionals and 39 (25%) paraprofessionals. During the review of the personnel records, the following deficiencies were noted:

Rule Found Deficient	Credential Validated	Personnel Record Number	Reviewer Notes
Medicaid IP Sec. 2: 221.804.C Act 806 of 2023 A.C.A. 9-28-130 (d)(1)(C)	CPR training - IP Acute	SR020485	No File Received
Medicaid IP Sec. 2: 215.220, 218.000 Act 806 of 2023 A.C.A 9-281302(d)(1)(C)	Professional License or Certificate - IP Acute	SR020492	No File Received
Medicaid IP Sec. 2: 241.100.B Act 806 of 2023 A.C.A. 9-28-1302(d)(1)(C)	Child Maltreatment Check - IP Acute	SR020487	Expired

Clinical Review Deficiencies:

AFMC was provided with the alleged victim's file for review.

The provider uploaded records which were then reviewed for compliance with licensure standards. Based on the review of clinical components of licensure requirements, the following deficiencies were noted:

Record Number	Rule	Reviewer Notes
RR0040322	Medicaid IP Sec. 2: 204.100	The Group Therapy Notes and the Family Therapy Notes did not document the setting in which the services were provided.
RR0040322	Medicaid IP Sec. 2: 204.100	The Individual Therapy Notes, Group Therapy Notes, and Family Progress Notes did not document the client's progress. However, the Group Therapy Notes for the month of May were documented in different forms and did have the patient's progress documented.
RR0040322	Medicaid IP Sec. 2: 218.100	The provider lacked evidence that the Individual Plan of Care was developed in consultation with the recipient and his or her

	Act 806 of 2023 A.C.A. 9-28-1302(b)(3)	parent(s), legal guardian(s), or others in whose care he or she will be released after discharged. The client's guardian stated in a phone interview that the facility never called her with any updates or changes to the client's Individual Plan of Care.
RR0040322	Medicaid IP Sec. 2: 218.200	The provider lacked evidence in the Individual Plan of Care of a description of the functional level of the client.

Summary of Findings and Resolution:

Below is a summary of the telephone interview with the guardian:

The guardian was contacted via telephone on June 3, 2025, at 8:30 a.m. by AFMC staff to interview her regarding the circumstances surrounding the complaint. She stated that when she picked up the client on Saturday, August 16, 2024, for a weekend home visit she immediately noticed her daughter visible, uncontrollable shaking in both hands, both legs, and her head. She also observed the client drooling excessively. She was told by staff this was normal due to the medications she had been placed on. She stated that the staff acknowledged at the time she was picking her daughter up that her daughter had these symptoms, but the staff did not state how long the symptoms had been going on. The guardian stated no one from the facility had contacted her to let her know the client had been exhibiting tremors, shaking, drooling, or any other side effects from the medication. The guardian stated that she had been informed that the medications were being administered to the client to help with [REDACTED]. The guardian stated the staff was angry that the client was trying to run off and the medications were being used to control the client and not proper oversight of the client. The guardian did receive enough medications to administer for the entire weekend pass, but she did not administer any of the medications to the client once they left the facility.

After leaving the facility, the guardian stated that the client continued to 'not act right' and she took her straight to her primary care physician at [REDACTED] in [REDACTED] where she saw the physician assistant. A [REDACTED] was drawn and found to be [REDACTED]. The clinic sent the client home with the guardian at that time and was told to go home and monitor the client for changes or worsening in symptoms. Later in the day, the physician assistant called the guardian and stated that she had consulted with a colleague regarding the client's condition and was recommending the guardian to take the client to the emergency room immediately. The guardian took the client to the emergency room at [REDACTED]. Upon arrival to the emergency room the client was found to have [REDACTED] which were diagnosed as [REDACTED]. [REDACTED] The client was placed on [REDACTED] for [REDACTED]. The client remained in [REDACTED] for approximately [REDACTED].

Once discharged from [REDACTED], the client was sent home with a [REDACTED] to prevent [REDACTED]. There was a concern initially about the client going home on [REDACTED], but her [REDACTED] stabilized in the hospital, so she did not require [REDACTED] upon discharge. The client had to check her [REDACTED] levels three to four times daily, which guardian states were traumatizing and had to check [REDACTED] due to [REDACTED] being way too high or dropping way too low. Upon discharge from the hospital, the client also had [REDACTED]. All issues have been resolved, and the client was discharged [REDACTED] from the [REDACTED].

The guardian did state that she was not involved in formulating the client's treatment plan or discharge plans. She stated that she had to initiate calls to the facility to inquire about what the treatment and discharge plans were. She stated the only calls she received from the facility were asking for consent for medication changes or when the client was misbehaving or getting in trouble. The guardian stated that one staff member even asked, "Do you realize she is a bully?", in reference to the client. That staff member put the guardian on the phone with the client so the guardian could "scold her for bad behavior." The guardian stated that she asked this staff member if they were aware that the client has a diagnosis of [REDACTED] which is the cause for the outburst of behaviors.

and the reason why the client is receiving treatment. The guardian stated the staff member told her they were not aware of the diagnosis. The guardian called the case manager to discuss this situation and was told by the case manager that those things (referring to the [REDACTED]) were on a need to know basis for staff.

Below is a summary of observations from the chart review of the alleged victim:

- The client was admitted to Habilitation Center LLC on [REDACTED] 2024, at [REDACTED], and was discharged on [REDACTED] 2024, after not returning from a [REDACTED] with the guardian.
- The client had multiple episodes documented in the nursing notes and staff observation notes detailing the client's behaviors that warrant the use of [REDACTED]. All documentation was completed thoroughly and accurately.
- On August 11, 2024, at 7:30 p.m., the night shift nurse (SR020522) documented "Patient on unit taking medications, this nurse noticed that patient's hands had tremors while trying to take medications." No follow up documentation was noted in the chart regarding tremors, nor was this documented as being reported to the physician. AFMC staff interviewed facility staff (SR020522) via phone, and they reported they had sent an email to the physician's nurse (SR020529) via email and that is the normal practice. AFMC staff interviewed SR020529, and she reported that she did not recall receiving an email and that if she did, she would have forwarded it to the physician's nurse that was providing care to the client. AFMC interviewed the physician's nurse (SR020530) that was overseeing the client, and she reported that she did not recall receiving any information about the tremors.
- AFMC requested to view the emails, and it was reported that 'corporate policy for email retention is 6 months, so anything older than that would have rolled off.'

Upon review of the physician orders, vital sign sheet, and laboratory values the following observations were noted:

- Medication Orders:
 - On May 23, 2024, at 3:43 p.m., the order was received to continue the following home medications:
 - [REDACTED]
 - [REDACTED]
 - On June 3, 2024, at 12:45 p.m., the order was received to [REDACTED].
 - On June 13, 2024, at 1:15 p.m., the order was received to [REDACTED].
 - On June 27, 2024, at 1:45 p.m., the following medication orders were received:
 - [REDACTED]
 - [REDACTED].
 - On July 2, 2024, at 12:30 p.m., the order was received to [REDACTED].
 - On July 12, 2024, at 1:45 p.m., the order was received to [REDACTED].
 - On July 26, 2024, at 2:45 p.m., the following medication orders were received:
 - [REDACTED]
 - [REDACTED].
- [REDACTED]:
 - [REDACTED] were drawn as per the physician's order and policy. All results were within the [REDACTED].
- Weight gain documentation:
 - The client's weight on admission on [REDACTED], 2024, was [REDACTED].
 - On June 2, 2024, the client's weight was [REDACTED].
 - On July 5, 2024, the client's weight was [REDACTED].
 - On August 1, 2024, the client's weight was [REDACTED].

- There was no documentation to address the client's weight gain except a physician order on July 7, 2024, at 2:30 p.m., the order was received to [REDACTED].
- Due to the weight gain AFMC staff reviewed the physical activity schedule provided to the clients as well as the daily schedule of what this client participated in. There were 20 days this client was not identified as being off the unit where appropriate physical activity could take place. Additionally, there is no indication of what is offered while clients are outside or in the gymnasium for activities. The facility staff stated they can only offer clients activities but cannot force them to participate. While onsite AFMC staff observed several groups of clients outside during activity time, but staff were not observed offering any activities but instead they were sitting with clients at picnic tables.
- While onsite, AFMC staff nurse looked at the documented monthly weights for the current clients on Penguin Hall. There were nine clients whose weights were documented on admission, three days after admission, and monthly thereafter. Of the nine clients the following were noted to have significant weight gains:
 - One client had a 22.4 pound weight gain in a 5 month period.
 - One client had a 15.2 pound weight gain in a 2 ½ month period.
 - One client had a 23.4 pound weight gain in a 10 month period.
 - One client had a 58.2 pound weight gain in a 22 month period.
- AFMC staff spoke with the Director of Nursing who provided four week rotating menus for breakfast, lunch, and dinner. The Director of Nursing explained that if a client receives an 1800 calorie diet the only change from a regular diet is smaller portion sizes and a different snack such as an apple instead of a prepackaged fudge round. Clients also are offered three snacks per day in addition to the three meals offered each day.

Additional statements:

- AFMC was unable to retrieve medical records from the PCP or [REDACTED] due to not having a release of information.
- The observations detailed above are not a direct regulation deficiency within the Licensure Standards for Inpatient Psychiatric Services for Under 21; however, they are significant to the complaint and client care.

Respectfully

Inspection of Care Team
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